Desert Orthopaedic Center

Surgery (Providers deposit)

Appointment: 04/10/2026 8:30 AM

Provider: Winder, Richard MD

Patient Information

Name:	John Slatauski	**Home Phone:**	(702) 955-5391
D.O.B:	09/25/1940	**Work Phone:**	
Sex:	Male	**Mobile Phone:**	(702) 482-5422
SSN:	XXX-XX-7976	**Email:**	jsrks@aol.com
MRN:	1741115	**Preferred Contact Method:**	Unspecified
PMS ID:	1741115	**Language:**	English
Marital Status:	Married	**Emergency Contact:**	Cindy Farinholt
Race:	White	**Emergency Contact Phone:**	702-286-1863
Ethnicity:	Not Hispanic or Latino	**Employer:**	
Address:	7862 Widewing Drive North Las Vegas, NV 89084	**Primary Care Provider:**	Bomberger, Shana MD
		Referring Providers:	Thompson, Aileen APRN Baldauf, John MD Winder, Richard MD

Primary Insurance Information

Carrier:	Veterans Community Care Network (VACCN) Triwest Military Payers	**Address:**	PO Box 108851 Florence, SC 295028851
Policy #:	1038307475		
Group ID/Name:	7346243588	**Phone Number:**	(877) 226-8749

Pharmacy Information

Name:	Mike O'Callaghan Military Medical Center		
Phone Number:	(702) 653-2273	**Address:**	4700 N Las Vegas Blvd, Unit 1300, FL 1 Las Vegas, NV 89115
Fax Number:			

DOC | DESERT ORTHOPAEDIC CENTER

Experience. Excellence.

(702) 731-1616

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Richard P. Winder, M.D.
Richard S. Woodworth, M.D.
Jessica R. Zarndt D.O.

Main Office	Northwest Office	Southwest Office	Henderson Office	Summerlin Office
2800 East Desert Inn Road Suite 100 Las Vegas, NV 89121	8402 West Centennial Parkway Las Vegas, NV 89149	8205 West Warm Springs Road Suite 250 Las Vegas, NV 89113	2930 West Horizon Ridge Parkway Suite 100 Henderson, NV 89052	8689 West Charleston Blvd. Suite 105 Las Vegas, NV 89117

FOR: John Statauski DOB: 9/25/40

ADDRESS: DATE: 4/9/26

Rx Home Health + Home PT
S/P L rotator cuff repair
4/10/26

ICD 10:

Refill: PRN: NR:

Refill: Times Dr.:

Dr. License No.: DISPENSE ONLY AS WRITTEN ☐ Rx

DOC-70 (Rev. 1/18) DEA No.:

Abbotts: (702) 456-8099 11/25 White (Patient) Yellow (Office)

Security Feature Placement Holder

Allergies
Reviewed January 21, 2026.
No known drug allergies

Medications
alfuzosin 10 mg Oral - tablet, extended release 24 hr
cyanocobalamin (vitamin B-12) 1,000 mcg Oral - tablet
ferrous sulfate 325 mg (65 mg iron) Oral - tablet
finasteride 5 mg Oral - tablet
rosuvastatin 10 mg Oral - capsule, sprinkle
sitagliptin 100 mg Oral - tablet

Medical History
Reviewed January 21, 2026.
Chronic anemia
H/O: hypertension
Type 2 diabetes mellitus
Other: PTSD

Musculoskeletal History
Reviewed January 21, 2026.

Surgical History
Reviewed January 21, 2026.
Other

Social History
Reviewed January 21, 2026.
Smoking status - Former smoker

Tobacco Use
Does not use vaping products
Does not use smokeless tobacco
Tobacco Product: None
Pneumonia vaccination administered or previously received

CC/HPI:
presents for preoperative exam. Patient is scheduled for a Left shoulder arthroscopy with rotator cuff repair, subacromial decompression and arthroscopic biceps tenodesis on 4/10/26 at IOS

Care timeline
Management history log

Summary:
Left Shoulder Rotator Cuff Tear with impingement, acromioclavicular arthritis, biceps tendinitis. Patient fell September 22, 2025 and has had significant shoulder pain since then. He had a cortisone injection that gave him a few days of relief. He has done 3 months of physical therapy which only worsened his symptoms. At this point I have recommended surgery. We will schedule the patient for left shoulder arthroscopy with rotator cuff repair, subacromial decompression, distal clavicle excision, and arthroscopic biceps tenodesis.
Patient has a history of diabetes and his hemoglobin A1c was elevated because of a medication miscommunication. He is resume that medication a month ago and is optimistic his hemoglobin A1c is better. He has a follow-up tomorrow with his primary care provider. His wife died a year ago. He is active and runs daily and wants to return to golf and bowling. He is a Vietnam war veteran and was exposed to agent orange there.

Findings:

Exam
Exam findings notable for the following:
Left shoulder exam: 90 degrees of active flexion. 4 out of 5 strength with resisted supraspinatus, 5 out of 5 with external rotation, 5 out of 5 with internal rotation. Positive Hawkins. Positive O'Brien's. Positive cross body adduction. Tenderness at the acromioclavicular joint and biceps groove.

Data Reviewed:
1 Review of the result(s) of each unique test (MRI Interpretation Shoulder (MRI Shoulder without contrast (imaging and outside report))) and 1 Independent interpretation of a test performed by another physician/other qualified health care professional (not separately reported) (MRI Interpretation Shoulder (MRI Shoulder without contrast (imaging and outside report)))

Tests

MRI Interpretation Shoulder

MRI: left MRI Shoulder without contrast (imaging and outside report)
Date: 01/08/2026

MRI of the left shoulder was reviewed, demonstrating the following findings: SDMI
IMPRESSION:
1. Moderate supraspinatus tendinosis with low-to-moderate grade bursal-sided critical zone tear measuring 5 mm in AP dimension. No full-thickness rotator cuff tear. Progressed since 2008 MRI.
2. Moderate infraspinatus tendinosis with bursal-sided tendon fraying. Progressed.
3. Moderate AC joint osteoarthrosis. Progressed

RW: Full-thickness tear at the insertion of the supraspinatus, impingement, biceps tendinitis.

Impression/Plan:

1. **Rotator Cuff Tear, Complete, Left**
   Complete rotator cuff tear or rupture of left shoulder, not specified as traumatic (M75.122)
   located on the left shoulder joint.
   Associated diagnoses: Bicipital Tendinitis, AC Arthritis, and Shoulder Impingement

   Plan: PT Rx - Rotator Cuff.
   Indication: Rotator Cuff Tear, Complete, Left - M75.122, M75.22, M19.012, M75.42

   Date of surgery: 04/10/2026

   Protocol: evaluate and treat per diagnosis/objective exam, left shoulder rotator cuff.
   Left Shoulder: Advance as tolerated.
   Recommend frequency of 2-3 times per week for 6-8 weeks.
   - Therapeutic Exercises: All exercises prn per therapist, Home program exercises, Strengthening exercises, and Stabilization training.
   - Manual Therapy: All exercises prn per therapist.
   - Modalities: All modalities prn per therapist.

   Provider: Alexander B Frandsen, PA-C
   Priority: normal

   Associated with:
   MRI Interpretation Shoulder

Staff:

Alexander B Frandsen, PA-C (Primary Provider)

Areli Sanchez

Richard P Winder, MD (Bill Under)

PRELIMINARY

Rehabilitation Following Arthroscopic Rotator Cuff Repair Medium to Large Tears

I. **Phase I - Immediate Post-Surgical Phase (Days 1-10)**

Goals: Maintain Integrity of the Repair
Gradually Increase Passive Range of Motion
Diminish Pain and Inflammation
Prevent Muscular Inhibition

Days One to Six:

- Abduction pillow brace
- Pendulum Exercises
- Active Assisted ROM Exercise (L-Bar)
    - ER/IR in Scapular Plane at 45 degrees of abduction (pain-free ROM)
  - Passive ROM
    - Flexion to tolerance (painful ROM)
    - ER/IR in Scapular Plane at 45 degrees of abduction (pain-free ROM)
- Elbow/Hand Gripping & ROM Exercises
- Submaximal Painfree Isometrics (initiate days 4-5)
    - Flexion with elbow bent to 90 degrees
    - External Rotation
    - Internal Rotation
    - Elbow Flexors
- Cryotherapy for Pain and Inflammation
    - Ice 15-20 minutes every hour
- Sleeping
    - Sleep in pillow brace

Days Seven to Ten:

- Continue use of pillow brace
- Pendulum Exercises
- Progress Passive ROM to Tolerance
    - Flexion to at least 115 degrees
    - ER in Scapular Plane at 45 degrees abduction to 20-25 degrees
    - IR in Scapular Plane at 45 degrees abduction to 30-35 degrees
- Active Assisted ROM Exercises (L-bar)
    - ER/IR in Scapular Plane at 45 degrees abduction
    - Flexion to Tolerance*
      *Therapist Provides Assistance by Supporting Arm (esp. with arm lowering)
- Continue Elbow/Hand ROM & Gripping Exercises
- Continue Isometrics (submaximal and subpainful)
    - Flexion with Bent Elbow
    - Extension with Bent Elbow
    - Abduction with Bent Elbow

    - ER/IR with Arm in Scapular Plane
    - Elbow Flexion
- Initiate rhythmic stabilization ER/IR at 45 degrees abduction
- Continue Use of Ice for Pain Control
    - Use Ice at least 6-7 times daily
- Sleeping

- Continue Sleeping in Brace until Physician Instructs

Precautions:

1. No Lifting of Objects
2. No Excessive Shoulder Extension
3. No Excessive Stretching or Sudden Movements
4. No Supporting of Body Weight by Hands
5. Keep Incision Clean & Dry

II. Phase II - Protection Phase (Day 15 – Week 6)

Goals: Allow Healing of Soft Tissue
Do Not Overstress Healing Tissue
Gradually Restore Full Passive ROM (Week 4-5)
Re-Establish Dynamic Shoulder Stability
Decrease Pain & Inflammation

Days 15 – 21:

- Continue Use of Sling or Brace (physician or therapist will determine when to discontinue)
- Passive Range of Motion to Tolerance
  - Flexion to 140-155 degrees
  - ER at 90 degrees abduction to at least 45 degrees
  - IR at 90 degrees abduction to at least 45 degrees
- Active Assisted ROM to Tolerance
  - Flexion (continue use of arm support)
  - ER/IR in Scapular Plane at 45 degrees abduction
  - ER/IR at 90 degrees Abduction
- Dynamic Stabilization Drills
  - Rhythmic Stabilization Drills
    - ER/IR in Scapular Plane
    - Flexion/Extension at 100 degrees Flexion and 125 degrees flexion
- Continue All Isometric Contractions
- Initiate scapular isometrics
- Continue Use of Cryotherapy as needed
- Continue All Precautions
  - No lifting
  - No excessive motion

Weeks 4 - 5:

- Patient should exhibit full passive range of motion by week 4
- Continue all exercises listed above
- Initiate ER/IR strengthening using exercise tubing at 0 degrees of abduction (use towel roll)
- Initiate Manual Resistance ER Supine in Scapular Plane (light resistance)
- Initiate Prone Rowing to Neutral arm Position

- Initiate prone shoulder extension
- Initiate ER strengthening exercises
- Initiate Isotonic Elbow Flexion
- Continue use of ice as needed
- May use heat prior to ROM exercises
- May use pool for light AROM exercises
- Rhythmic stabilization exercises (flexion 45, 90, 125 degrees) (ER/IR)

Weeks 5 – 6:

- May use heat prior to exercises
- Continue AAROM and Stretching exercises
  - Especially for movements that are not full

- Shoulder flexion
- ER at 90 degrees abduction
- Initiate Active ROM Exercises
  - Shoulder Flexion Scapular Plane
  - Shoulder Abduction
- Progress Isotonic Strengthening Exercise Program
  - ER Tubing
  - Sidelying IR
  - Prone Rowing
  - Prone Horizontal Abduction (bent elbow)
  - Biceps Curls (isotonics)

Precautions:

1. No Heavy Lifting of Objects
2. No excessive behind the back movements
3. No Supporting of Body Weight by Hands & Arms
4. No Sudden Jerking Motions

III. **Phase III – Intermediate Phase (Weeks 7-14)**

Goals: Full Active ROM (Week 8-10)
Maintain Full Passive ROM
Dynamic Shoulder Stability
Gradual Restoration of Shoulder Strength
Gradual Return to Functional Activities

Week 7:

- Continue Stretching & PROM (as needed to maintain full ROM)
- Continue Dynamic Stabilization Drills
- Progress Strengthening Program
  - ER/IR Tubing
  - ER Sidelying
  - Lateral Raises*
  - Full Can in Scapular Plane*
  - Prone Rowing
  - Prone Horizontal Abduction
  - Prone Extension
  - Elbow Flexion

  - Elbow Extension

*Patient must be able to elevate arm without shoulder or scapular hiking before initiating isotonics; if unable, continue glenohumeral joint exercises.

Week 8:

- Continue all exercise listed above
- If physician permits, may initiate Light functional activities

Week 10:

- Continue all exercise listed above
- Progress to Fundamental Shoulder Exercises
- Therapist may initiate isotonic resistance (1 lb wt.) during flexion and abduction*
  - *If non-painful normal motion is exhibited!

Weeks 11-14:

- Progress all exercises
  - Continue ROM and flexibility exercises

- Progress strengthening program (increase 1 lb/10 days *non-painful)

IV. **Phase IV – Advanced Strengthening Phase (Weeks 15 - 22)**

Goals: Maintain Full Non-Painful ROM
Enhance Functional Use of UE
Improve Muscular Strengthen & Power
Gradual Return to Functional Activities

Week 15:

- Continue ROM & Stretching to maintain full ROM
- Self Capsular Stretches
- Progress Shoulder Strengthening Exercises
  - Fundamental Shoulder Exercises
- Initiate Interval Golf Program (if appropriate)

Weeks 20- 22:

- Continue all exercises listed above
- Progress Golf Program to playing golf (if appropriate)
- Initiate Interval Tennis Program (if appropriate)
- May Initiate Swimming

V. **Phase V – Return to Activity Phase (Weeks 23 - 36)**

Goals: Gradual Return to Strenuous Work Activities
Gradual Return to Recreational Sport Activities

Week 23:

- Continue Fundamental Shoulder Exercise Program (at least 4 times weekly)
- Continue Stretching, if motion is tight
- Continue Progression to Sport Participation

KEW/JRA:12/08

Department of Veterans Affairs

REQUEST FOR SERVICES (RFS) FORM

PREVIOUS AUTHORIZATION NUMBER: VA0052658146

TODAY'S DATE (MM/DD/YYYY): 04/09/26

NOTE: The Request for Services (RFS) Form 10-10172 must be submitted via an approved method (HSRM, Electronic fax, Direct Messaging, traditional fax, or mail). Completion of this form is REQUIRED and MUST BE SIGNED by the requesting provider for further care to be rendered to a Veteran patient.

SECTION I: VETERAN INFORMATION

1. VETERAN'S LEGAL FULL NAME (First, MI, Last): Slatauski, John 7976
2. DOB (MM/DD/YYYY): 09/25/40
3. VA FACILITY:
4. VA LOCATION:

SECTION II: ORDERING PROVIDER INFORMATION

5. REQUESTING PROVIDER'S NAME: Richard P. Winder
6. NPI #: 1760610810
7. SPECIALTY: ortho
8. OFFICE NAME & ADDRESS: Desert Orthopaedic Center 2800 E. Desert Inn Rd #100 Lv. NV 89121
9. SECURE EMAIL ADDRESS: N/A
10. PHONE NUMBER: 702-731-4416
11. FAX NUMBER: 702-734-4900
12. ☐ INDIAN HEALTH SERVICES (IHS) PROVIDER?

SECTION III: TYPE OF CARE REQUEST

13. PLEASE INDICATE CLINICAL URGENCY (*Urgent care is only applicable for requests that require less than 3 days to process. If care is needed within 48 hours or if Veteran is at risk for Suicide/Homicide, please call the VA directly on the same day as completed RFS form submission. Do NOT mark urgent for administrative urgency*):
☐ ROUTINE ☒ URGENT

14. DIAGNOSIS (*ICD-10 Code/Description*): Left shoulder rotator cuff tear M75.122
15. DATE OF SERVICE (MM/DD/YYYY) &/OR ANTICIPATED LENGTH OF CARE: 4/12 or 4/13 x 2-3 months
16. CPT/HCPCS CODE &/OR DESCRIPTION OF REQUESTED SERVICES (*Include units/visits, add second list page, if needed*):
17. HOW MANY VISITS HAVE OCCURRED SO FAR? (*If known*): Ø
18. IS THIS A REFERRAL TO ANOTHER SPECIALTY? ☐ YES (*If "YES," please fill out the Servicing Provider/Specialty information below*) ☒ NO
19. SERVICING PROVIDER'S NAME:
20. NPI #:
21. SPECIALTY:
22. OFFICE NAME & ADDRESS:
23. SECURE EMAIL ADDRESS:
24. PHONE NUMBER:
25. FAX NUMBER:

SECTION IV: TYPE OF SERVICE REQUESTED

26. OUTPATIENT CARE: ☐ PT ☐ OT ☐ SPEECH THERAPY
FREQUENCY & DURATION:
27. SURGICAL PROCEDURE: ☐ INPATIENT ☐ OUTPATIENT
FACILITY NAME:
28. ☐ IN-OFFICE PROCEDURE
29. INPATIENT CARE: ☐ LTACH ☐ ACUTE REHAB ☐ BH
30. ☐ ADDITIONAL OFFICE VISITS (*List if needed*):
31. ☐ EXTENSION OF VALIDITY DATES
32. ☐ EMERGENCY ROOM CARE
33. ☐ LABS (*If done outside of office, please provide facility above*)
34. ☐ RADIOLOGY/IMAGING (*If done outside of office, please provide facility above*)
35. ☐ PRE-OPS LABS ☐ CHEST XRAY ☐ EKG ☐ OTHER:
36. JUSTIFICATION FOR REQUEST (*To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested*).

S/P Left shoulder arthroscopy & Rotator cuff repair on 4/10/26. will need assistance c̄ ADLs & will need home PT.

VETERAN'S LEGAL FULL NAME *(First, MI, Last)*: John Slatauski 7976

SECTION V: GERIATRICS AND EXTENDED CARE SERVICES *(If applicable)*:

37. ☐ COMMUNITY ADULT DAY HEALTH CARE ☐ COMMUNITY NURSING HOME ☐ HOMEMAKER/HOME HEALTH AIDE
☐ HOME INFUSION ☐ HOSPICE/PALLIATIVE CARE ☐ RESPITE
☐ SKILLED HOME HEALTH CARE ☐ OTHER: _____

FREQUENCY & DURATION: _____

38. JUSTIFICATION FOR REQUEST *(To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).*

SECTION VI: HOME OXYGEN INFORMATION *(If applicable)*:

39. PAO2 AT REST:	40. O2 SAT AT REST:	41. OXYGEN FLOW RATE:

42. EXTENT OF SUPPORT *(Continuous, Intermittent, Specific Activity)*:

43. OXYGEN EQUIPMENT *(Stationary/Portable)*:

44. DELIVERY SYSTEM *(Cannula, Mask, Other)*:

SECTION VII: DME & PROSTHETICS INFORMATION *(If applicable)*:

45. HCPS FOR THE ITEM(S) BEING PRESCRIBED:

46. BRAND, MAKE, MODEL, PART NUMBERS:

47. MEASUREMENTS:

48. QUANTITY:	49. ICD-10:	50. PROVISIONAL DIAGNOSIS:

51. DELIVERY/PICKUP OPTIONS:
☐ DELIVER TO ORDERING PROVIDER'S ADDRESS ☐ VETERAN WILL PICKUP AT THE VA MEDICAL CENTER
☐ DELIVER TO COMMUNITY VENDOR FOR DELIVERY & SETUP FOR DME ☐ DELIVER TO VETERAN'S HOME

SECTION VIII: DURABLE MEDICAL EQUIPMENT *(DME)* EDUCATION & TRAINING *(If applicable)*:

Please see DME Requirements/Pharmacy Requirements - Community Care (va.gov) **for URGENT DME requests.**

NOTE: Failure to thoroughly complete the RFS for DME will result in delayed patient care & prevent the VA from DME fulfillment.

52. BEFORE DME WILL BE ISSUED, EDUCATION, TRAINING, &/OR FITTING OF DME *(as applicable for the specific DME being ordered)* TO THE VETERAN MUST BE COMPLETE. PLEASE INDICATE WHETHER THE FOLLOWING HAS BEEN COMPLETED FOR THE VETERAN.

NOTE: If not completed, DME will be mailed to requesting provider's address to coordinate an alternative time for proper instruction on DME use.

A. EDUCATION:	☐ YES	☐ NO	
B. TRAINING:	☐ YES	☐ NO	☐ N/A
C. FITTING:	☐ YES	☐ NO	☐ N/A

53. JUSTIFICATION FOR REQUEST *(To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).*

VETERAN'S LEGAL FULL NAME *(First, MI, Last)*: John Vlatauski

SECTION IX: THERAPEUTIC FOOTWEAR ASSESSMENT INFORMATION *(If applicable)*

54. FILL OUT THE INFORMATION BELOW *(If applicable)*:

☐ LEFT FOOT ☐ RIGHT FOOT ☐ BILATERAL

☐ PREFABRICATED THERAPEUTIC FOOTWEAR

☐ CUSTOM THERAPEUTIC FOOTWEAR

NOTE: For prescription of therapeutic footwear for severe or gross foot deformity which cannot be accommodated with conventional footwear.

DESCRIBE FOOT DEFORMITY AND ADDITIONAL DETAILS:

NOTE: For prescription of therapeutic footwear due to disease pathology resulting in neuropathy or peripheral artery disease.

55. CHECK APPROPRIATE DIABETIC/AMPUTATION RISK SCORE:

☐ **RISK SCORE 2:** PATIENT DEMONSTRATED SENSORY LOSS *(inability to perceive the Semmes-Weinstein 5.07 monofilament)*, DIMINISHED CIRCULATION AS EVIDENCED BY ABSENT OR WEAKLY PALPABLE PULSES, FOOT DEFORMITY, OR MINOR FOOT INFECTION, & A DIAGNOSIS OF DIABETES.

☐ **RISK SCORE 3:** PATIENT DEMONSTRATED PERIPHERAL NEUROPATHYWITH SENSORY LOSS *(i.e., inability to perceive the Semmes-Weinstein 5.07 monofilament)*, AND DIMINISHED CIRCULATION, AND FOOT DEFORMITY, OR MINOR FOOT INFECTION & A DIAGNOSIS OF DIABETES, OR ANY OF THE FOLLOWING BY ITSELF: (1) PRIOR ULCER, OSTEOMYELITIS OR HISTORY OF PRIOR AMPUTATION; (2) SEVERE PERIPHERAL VASCULAR DISEASE *(PVD) (intermittent claudication, dependent rubor with pallor on elevation, or critical limb ischemia manifested by rest pain, ulceration or gangrene)*; (3) CHARCOT'S JOINT DISEASE WITH FOOT DEFORMITY; & (4) END STAGE RENAL DISEASE.

NOTE: Only patients who are experiencing medical conditions noted in the risk scores can be prescribed therapeutic/diabetic footwear.

***ATTESTATION:** I do hereby attest that the forgoing information is true, accurate, & complete to the best of my knowledge & I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability.

I do hereby acknowledge that VA reserves the right to perform the requested service(s) if the following criteria are met: (1) The patient agrees to receive services from VA (2) Service(s) are available at VA facility & are able to be provided by the clinically indicated date (3) It is determined to be within the patients best interest. Upon completion of the requested service(s), VA will provide all resulting medical documentation to the ordering provider. If all criteria listed are not true & VA agrees the service(s) are clinically indicated, VA will provide a referral for services to be performed in the community.

I do hereby attest that upon receipt of order/consult results, I will assume responsibility for reviewing said results, addressing significant findings, & providing continued care.

56. REQUESTING PROVIDER SIGNATURE *(Required)*:

57. TODAY'S DATE *(MM/DD/YYYY)*: 04/09/2024

To facilitate timely review of this request, the most recent office notes & plan of care must accompany this signed form.

For more information please visit: https://www.va.gov/COMMUNITYCARE/providers/Care_Coordination.asp.

VA Community Care Medical Policies describe standard VA health care benefit for services and procedures that community providers may recommend as necessary for a Veteran. Prior to providing care, providers should use the Community Care Medical Policies as a reference when determining if a Veteran meets VA clinical criteria. When additional services are requested, Community Care Medical Policies will be used to determine approval by a clinical reviewer. Community Care Medical Policies & supporting information can be found at: https://www.va.gov/COMMUNITYCARE/providers/Medical-Policy.asp

U.S. Department of Veterans Affairs

VA Form 10-7080 - Approved Referral For Medical Care

Veteran Name: Slatauski, John
Veteran ICN: 1010393481V620419
Veteran EDIPI: 1038307475
Veteran Date of Birth: 1940-09-25
Veteran Preferred Name:
Pronoun:
Pronoun Description:
Veteran Address: 7862 WIDEWING DR
N LAS VEGAS, NV 89084
Veteran Phone Number: +1 (702) 4825422
Veteran Mobile Phone Number (If Known): +1 (702) 4825422
Veteran Business Phone Number (If Known): (702)482-5422
Veteran Email Address (If Known): JSRKS@AOL.COM

Referral Number: VA0052658146
Priority: Routine
Referral Issue Date: 2025-10-20
Expiration Date: 2026-12-17
First Appointment Date: 2025-12-17

Referring VA Facility: Las Vegas VA Medical Center
VA Telephone Number: (702) 791-9066
VA Fax Number: 702-791-9216 /702-791-9217/ DME 702-791-9224/ RFS 702-791-9225

Referring Provider: AILEEN THOMPSON
Referring Provider NPI: 1285098053

Initial Community Care Provider/Facility: Desert Orthopaedic Center Ltd
Initial Provider Location: Desert Orthopaedic Center Ltd-8402 Centennial Pkwy, Las Vegas, NV, 89149-193200000X
Provider Name (If known): Desert Orthopaedic Center LTD
Provider Phone: 9999999999
Provider Fax: na
Community Provider NPI: 1548235302
Caregiver Type:
Caregiver Details:

Any claim related to this episode of care **MUST INCLUDE THE APPROVED REFERRAL NUMBER** as the Referral Number or Prior Authorization number.

Please see below for Additional VA Referring Facility Information, Billing Information and Appointment/Provider Information.

Pertinent Clinical Information

Please view the Clinical Information in the VA Order section for more information related to the Original VA Order Reason for Request.

Please refer to your referral package for additional clinical consult information.

Provisional Diagnosis: M25512 Pain in left shoulder

Services Authorized

The VA Order Reason for Request is the official clinical order. The scope of services and number of visits associated with the medical care for this authorization can be found below. Necessary services that are not included must be requested using the Request for Services procedures. Please visit the VHA Storefront www.va.gov/COMMUNITYCARE/providers/index.asp for additional resources and requirements.

Service Requested: Orthopedics General SEOC 1.0.18

Category of Care: ORTHOPEDIC

Procedural Overview - Standardized Episode of Care (SEOC)

Orthopedics General SEOC 1.0.18 Duration: 365 Days

Description:

This authorization covers services associated with the specialty(s) identified for this episode of care, including all medical care listed below relevant to the referred care specified on the consult/referral order.

NOTE: Radiation therapy is NOT included in this episode of care approval. Radiation therapy requires a Request for Services (RFS) with supporting documentation to allow for clinical review and consideration of a separate referral specifically for radiation therapy.

NOTE: Pain neuromodulation devices for pain management should use the Pain Neuromodulation Devices Standardized Episode of Care (SEOC).

No.	Service/Procedure	Number Of Visits Authorized
1	Initial outpatient evaluation, treatment, and follow-up visits for the referred condition indicated on the consult/referral order	999
2	Diagnostic imaging relevant to the referred condition on the consult/referral order	999
3	Labs and pathology relevant to the referred condition on the consult/referral order	999
4	Diagnostic studies relevant to the referred condition on the consult/referral order including but not limited to: electromyography (EMG), nerve conduction study (NCS)	999
5	Anesthesia consultation related to the procedure or surgery relevant to the referred condition on the consult/referral order	999
6	Pre-procedure medical and basic cardiac clearance provided by the specialist's office, as clinically indicated (including H+P, labs, EKG, CXR, and echo when not done previously at the VA within the required procedure timeframe) NOTE: cardiac testing or evaluation outside of the above CXR, EKG, and echo, when clinically indicated, will require an RFS for a cardiology referral	999
7	Procedures performed by the orthopedic surgery provider relevant to the referred condition on the consult/referral order including but not limited to: injections, biopsies, fracture management (including casting), arthroscopies, arthroplasties	999
8	Inpatient or observation admission for orthopedic procedure and/or procedure-related complications, if indicated NOTE: Notify the referring VA of admission status to initiate and facilitate care coordination and discharge planning	999

9	Outpatient physical therapy clinic visits (excludes in-home services), as clinically indicated, not to exceed 15 visits NOTE: RFS is required to request additional visits with supporting medical documentation	15
10	Outpatient occupational therapy clinic visits (excludes in-home services), as clinically indicated, not to exceed 15 visits NOTE: RFS is required to request additional visits with supporting medical documentation	15

Additional Information:

REFER ALL QUESTIONS RELATED TO THIS APPROVAL TO THE ISSUING VA OFFICE

Referring VA Facility: Las Vegas VA Medical Center
Station Number: 593
Telephone Number: (702) 791-9066
Address: 6900 North Pecos Road NORTH LAS VEGAS NV 89086
Referring Provider: AILEEN THOMPSON
Referring Provider NPI: 1285098053
Unique Consult No: 593_6419122
Program Authority: Authorized/Pre-authorized VA Referral (not otherwise specified) - 1703
Affiliation: Triwest
Network: CC Network 4

Appointments/Providers Assigned to the Referral

Provider/Facility Name	Provider/Facility Location	Appt Date	Appt Time	Telephone #	Fax #
Desert Orthopaedic Center Ltd NPI 1548235302	Desert Orthopaedic Center Ltd-8402 Centennial Pkwy, Las Vegas, NV, 89149-193200000X	2025-12-17	09:40 PST	999-999-9999	na

Additional Service Information

Request for Services (RFS)
A Request for Services (RFS) is a provider-generated request for new or additional care outside the scope of the current approved referral/authorization. Provider should always submit the RFS directly to the authorizing VAMC, preferably via the HSRM portal. Providers should always submit an RFS on the same day it is determined it's needed and before delivering care, unless it is emergent care. In that case, the RFS can be submitted simultaneously.

How to Submit an RFS
VA prefers providers submit an RFS via the HSRM portal, available on the VA's website
- Go to the VA Storefront at https://www.va.gov/COMMUNITYCARE/providers/Care-Coordination.asp#RFS
  - Navigate to the link to the RFS form at the bottom of the section

Medical Records and Documents Requirements

Medical Records and documentation are required for all provider services. Providers are required to submit Medical documentation directly to the authorizing VAMC, preferably via upload to HSRM.

Billing and Other Referral Information

Submitting Claims

ANY CLAIMS RELATED TO THIS EPISODE OF CARE MUST BE SUBMITTED TO TRIWEST OR DELTA DENTAL AS APPROPRIATE AND INCLUDE THE APPROVED REFERRAL NUMBER.

Methods to submit claims:
Electronic Data Interchange (EDI):
Payer ID for Medical – TWVACCN
Payer ID for Dental – CDCA1

More information on how to submit claims can be found by visiting
https://www.va.gov/COMMUNITYCARE/revenue-ops/Veteran-Care-Claims.asp

Precertification

The Standardized Episode of Care (SEOC) referral you have accepted does not include services that require third-party payer (TPP) precertification.

Clinical Determinations and Indications

VA Clinical Determinations and Indications (CDIs) describe standard VA health care benefits for services and procedures that community providers may recommend as necessary for a Veteran. Prior to providing care, providers should use the CDIs as a reference when determining if a Veteran meets VA clinical criteria. When additional services are requested, CDIs will be used to determine approval by a clinical reviewer. CDIs and supporting information can be found at: https://www.va.gov/COMMUNITYCARE/providers/info-CDI.asp

Medical Devices

As part of furnishing the services authorized on the approved referral, you may recommend that medical devices, adaptive equipment, or other items be provided for the treatment or rehabilitation of the Veteran's medical condition. In such instances, you must complete a Request for Services (RFS) as outlined above.
https://www.va.gov/COMMUNITYCARE/providers/Care-Coordination.asp#RFS

The RFS must (1) indicate that the condition for which the item is being prescribed is within the scope of the authorized services on the approved referral, (2) describe the item or service being prescribed with as much specificity as possible (including manufacturer and model, needed custom measurements, size), and (3) provide a brief but thorough plain-language explanation of how the item or service will serve the treatment or rehabilitation needs of the Veteran. All parts of the RFS form must be filled out completely.

VA staff will determine whether the prescribed item or service is one that VA is authorized to purchase. If the item is not something that we are authorized to purchase, VA staff will work with you to identify an alternative that will best meet the Veteran's clinical needs. In making this determination, VA staff will consider your clinical judgment and expertise and work with you to resolve outstanding issues quickly.

In the event of an urgent or emergent need, you may provide medical devices, necessary items or services prescribed to Veterans receiving care in the community for urgent or emergent conditions at the time of healthcare service delivery or soon thereafter. Urgent or emergent DME or Medical Devices may include, but are not limited to splints, crutches, canes, slings, soft collars, walkers, and manual wheelchairs. DME dispensed directly to the Veteran based on an urgent or emergent need will be paid by the CCN Third Party Administrator. Claims for these items should be submitted using the process outlined in the **Submitting Claims** section of this form.

Pharmacy

Drug Safety and Administration Requirements

- Must follow the VA National Protocol and clinical guidance for Esketamine or Ketamine administration. Ketamine treatments for mental health or for pain are not approved under a CCN referral. Prior to administration it is required the ordering and administering providers review the VA Protocol and Clinical Guidance found through the VA Formulary Search Tool available here:
- https://www.pbm.va.gov/apps/VANationalFormulary/

Must follow the VA Opioid Safety Guidelines and complete the required Opioid Safety Training found here:

- https://www.pbm.va.gov/apps/VANationalFormulary/
- https://www.va.gov/COMMUNITYCARE/providers/EDU-Training.asp

CVS Caremark is the retail pharmacy network for Veterans' immediately needed or Urgent/Emergent prescriptions.

Immediate need prescriptions:

- Must follow the VA Urgent/Emergent Formulary which can be found at http://www.pbm.va.gov/PBM/nationalformulary.asp
- Prescription can only go up to a 14-day supply. No refills of the immediate need medication may be authorized.
- Only a seven-day supply for opioids, or up to the opioid prescribing limit allowed by State—whichever is less—may be authorized.

Immediate need prescription extending past 14 days:

- The provider will need to send second prescription (beyond 14 days) to the referring VA medical facility's pharmacy for prescription fulfillment services.

Routine/maintenance prescriptions:

- Must be sent to the referring VA medical facility's pharmacy

If you do not have the ability to electronically submit prescriptions to pharmacies, please contact the Community Care representative at the referring VA medical facility for their pharmacy fax number. Please refer to: https://www.va.gov/COMMUNITYCARE/providers/Service-Requirements.asp for additional instructions related to prescriptions.

Clinical Information on the VA Order

Reason for Request:

Orthopedic Outpatient Consult
ORTHOPEDICS ROUTINE CONSULT
Reason for Consultation:
Request follow-up, evaluation and treatment for 3-week history of left shoulder pain status post mechanical fall. Patient reports pain at the lateral deltoid, there is a soft tissue contusion noted, full range of motion. Patient reports intermittent paresthesias to the left upper extremity since the accident. He denies any previous surgery to the extremity. Distal pulse intact. Brisk capillary refill distally.
Bilateral grip strength is equal and strong.
Plain film x-ray as follows:
SHOULDER LEFT 2 OR MORE VIEWS (RAD Detailed) CPT:73030
CPT Modifiers : LT LEFT SIDE
Reason for Study: injury 3 weeks ago, persistent pain
Clinical History:
176.4 lb [80.01 kg] (09/09/2025 08:24)
Report Status: Verified Date Reported: OCT 13, 2025
Date Verified: OCT 13, 2025
Verifier E-Sig:

Report:
EXAMINATION: Left shoulder radiographs 4 views
HISTORY: Injury 3 weeks ago, persistent pain.
COMPARISON: None.
FINDINGS: 4 views of the left shoulder are obtained.
Bones: No acute fracture is seen in the left shoulder. Joint spaces: Moderate osteoarthritic degenerative changes of the left acromioclavicular joint with osteophytes. Tiny osteophytes at the glenoid. No dislocation. Hardware: No hardware is seen in the left shoulder. There are surgical clips in the region of the gastroesophageal junction. Alignment: Normal. Soft tissues/visualized lungs: Calcified plaque of the thoracic aorta.
Impression:
No acute fracture or dislocation is seen in the left shoulder.
Osteoarthritic degenerative changes in the left shoulder.

**This 10-7080 - Approved Referral For Medical Care was generated on 12/09/2025 changes made to the referral after this date are not reflected on the form.